

HOW TO OPEN AN IN-HOME CARE PROVIDER IN SOUTH CAROLINA

*A plain-language, step-by-step guide to starting a licensed non-medical home care agency in South Carolina
— written for real people, not lawyers or consultants.*

What this is. South Carolina licenses non-medical home care as an In-Home Care Provider under Regulation 60-122 (formerly 61-122), adopted under the Licensure of In-Home Care Providers Act (S.C. Code § 44-70-10 et seq.) and administered by the Department of Public Health (DPH), Bureau of Health Facilities Licensing. This guide walks the whole path — from deciding whether you need the license, through the application and screening rules, to getting paid and staying compliant.

How to use it. Read Parts 1 through 4 before you spend real money — they explain what the license authorizes and how to get it, including South Carolina’s distinctive drug-testing and background rules. Parts 5 and 6 are about revenue: where clients come from and how you get paid, including the Community Choices Waiver and the state’s mandated AuthentiCare electronic visit verification. Parts 7 through 9 keep you compliant. The two worked examples show how it comes together in Columbia and Charleston, and the directory, glossary, and references at the end are your quick-reference desk. Verify every fee, figure, and phone number before you rely on it — requirements change.

The Path at a Glance

Opening an In-Home Care Provider in South Carolina follows a clear sequence. None of it is complicated on its own; the work is doing each step in order and keeping the paper.

1. **Confirm you need the license.** If you will provide hands-on personal care for two or more individuals, you must be licensed as an In-Home Care Provider.
2. **Form the business.** Register with the SC Secretary of State, get an EIN, open a business bank account, and secure a physical business address.
3. **Write your policies.** Adopt a policy and procedure manual built to Regulation 60-122 (this product is that manual).
4. **Line up insurance.** Obtain general and professional liability coverage and workers' compensation.
5. **Apply to DPH.** Submit the In-Home Care Provider application, oath, policies, administrator resume, and fee to the Bureau of Health Facilities Licensing.
6. **Pass screening set-up.** Put your background-check, abuse-registry, drug-testing, and TB-screening processes in place before any caregiver has client contact.
7. **Survey and license.** DPH reviews your application, processes background checks, and may conduct an on-site survey before issuing the license.
8. **Enroll to get paid.** Set up private pay and, if serving Medicaid, enroll with SC Healthy Connections and the Community Choices Waiver and set up AuthentiCare EVV.
9. **Open and stay compliant.** Serve clients within scope, keep records, report as required, and maintain your license.

Part 1 — What a South Carolina In-Home Care Provider Is

An In-Home Care Provider (IHCP) is a business that, for compensation, provides non-medical care in a client's home. South Carolina defines in-home care (S.C. Code § 44-70-20) as care that primarily assists a client with an activity of daily living or a personal, non-medical need — and that does not require the continuing attention of licensed medical personnel.

What you can do

- Assist with activities of daily living: walking, getting in and out of bed, bathing, dressing, grooming, feeding, using the toilet, and preparing special diets
- Homemaker services: light housekeeping, laundry, meal preparation, shopping, and errands
- Companion and sitter services: supervision, conversation, activities, and escorting to appointments
- Supervise self-administered medication — reminding and helping a client take their own medication

What you cannot do

You do not provide skilled nursing, therapy, injections, wound care, or medication administration. Those require a separate license — skilled home health is licensed under Regulation 61-77, which is a different and more involved license with a registered-nurse clinical director and Medicare Conditions of Participation. If your clients will need skilled care, that is a different business.

The two-or-more rule

If you provide hands-on personal care (such as bathing or dressing) for **two or more** individuals, South Carolina requires you to be licensed as an In-Home Care Provider. Serving a single client under a private arrangement is generally exempt — but the moment you take a second hands-on client, you need the license. Verify your situation with DPH.

Part 2 — Getting Licensed (Regulation 60-122)

You apply to the South Carolina Department of Public Health (DPH), Bureau of Health Facilities Licensing. The application is document-driven: DPH wants to see that your business, policies, insurance, and screening processes are in place, and it may conduct an on-site survey before issuing the license.

What the application package includes

- The DPH In-Home Care Provider application form, completed accurately, including the applicant's oath that the contents are true and that the applicant will comply with Regulation 60-122 — signed by the owner(s), or by two officers if a corporation
- A non-refundable application fee (verify the current amount with DPH; recent third-party guidance cites approximately \$1,000)
- Your policy and procedure manual built to Regulation 60-122
- The administrator's resume showing the required qualifications
- Evidence of general and professional liability insurance
- Your client service agreement, service (care) plan, and incident report forms
- Proof of business registration and your physical business address

What DPH does next

DPH reviews your application and policies, processes the required background checks, and may schedule an on-site survey of your business office and records. Processing time is not fixed in the regulation; recent anecdotal reports suggest one to three months. There is no Certificate of Need for an in-home care provider, and unlike a facility there is no building or fire-safety inspection of client homes — the review is of your business, records, and compliance.

Do not operate before you are licensed

If you provide in-home care for two or more clients before DPH issues your license, you are operating unlawfully and DPH can order you to cease operations immediately. Get licensed first.

Part 3 — Governance, Staffing, Screening & Training

South Carolina's staffing rules (Regulation 60-122, Section 300) are stricter than many states — especially on drug testing. Build these processes before you hire, because every one of them must be done before a caregiver has contact with a client.

Your administrator

Your administrator must be at least 21 years old, hold a high school diploma or GED, and have at least one year of supervisory or administrative experience in home care or a related field. The administrator is in charge of all agency functions and is your point of contact with DPH.

Screening every caregiver — before client contact

- **State and national criminal background checks** (S.C. Code § 44-7-2910) — fingerprint-based checks through SLED and the FBI for owners, the administrator, and all direct-care staff.
- **Adult-abuse registry coordination** under the Omnibus Adult Protection Act (§ 43-35-10) before employment, contracting, or referral.
- **A pre-employment drug test and a written random drug-testing policy** using at minimum a five-panel screen for cannabis, cocaine, amphetamines, opiates, and phencyclidine.
- **No drug-related convictions** (or no-contest pleas) within ten years of providing in-home care.
- **Tuberculosis screening** using a TB risk assessment and, where indicated, a BAMT or other approved test.
- **Driver checks** where transportation is part of the job — a valid license, proof of insurance, and verification of the caregiver's official driving record.

Training

Train every caregiver, by appropriate resources, before client contact and on a continuing basis. Training must cover client rights and the prevention, detection, and reporting of abuse and neglect; confidentiality; assistance with activities of daily living; infection control; client safety and emergency and disaster procedures; supervising self-administered medication; and care specific to the client's condition, such as Alzheimer's disease, related dementia, and cognitive disabilities. Document orientation, competency, and continuing training for each caregiver.

Part 4 — Setting Up the Business

- **Register the business** with the South Carolina Secretary of State and choose your structure (most agencies form an LLC).
- **Get an EIN** from the IRS and open a separate business bank account.
- **Secure a physical business address** — a physical street location, not a post office box — where you keep records available for DPH survey.
- **Buy insurance** — general and professional liability, plus workers' compensation once you have employees.
- **Set up payroll** to employ caregivers as W-2 employees with proper tax withholding.
- **Adopt your policies** — the Regulation 60-122 policy and procedure manual — and your forms.

Employees, not contractors

Run your caregivers as W-2 employees. Misclassifying caregivers as 1099 independent contractors to avoid payroll taxes, overtime, and workers' compensation is a common and costly mistake, and it does not fit the control an agency exercises over its caregivers.

Part 5 — Finding Clients

Your license lets you operate; relationships fill your schedule. Build referral relationships with the people who meet your future clients at decision points — and do it lawfully, never paying for referrals of publicly funded clients.

- Hospital discharge planners and case managers sending patients home who need help
- Skilled nursing and rehabilitation facilities discharging residents
- Assisted living communities whose residents need one-to-one help
- Hospices needing extra in-home support hours
- Elder-law attorneys and geriatric care managers guiding families
- The aging network — Area Agencies on Aging and the SC Department on Aging
- Community Choices Waiver case managers, once you are enrolled

The companion product, The Referral System — South Carolina Edition, gives you a verified directory and metro-by-metro target lists to build this network.

Part 6 — Getting Paid

In-home care is paid several ways in South Carolina. Know each before you build your budget, and verify current rates with the payer — published rates change and are not reproduced here.

Private pay and insurance

Many clients pay privately, by the hour, and some use long-term care insurance or veterans' benefits. Private pay is the simplest to start with and does not require Medicaid enrollment or EVV.

South Carolina Medicaid — Healthy Connections

To serve Medicaid clients, enroll as a provider with South Carolina Healthy Connections (administered by the SC Department of Health and Human Services, SCDHHS). The main path for non-medical personal care is the Community Choices Waiver, one of the state's Community Long Term Care (CLTC) waivers for older adults and adults with disabilities who need help to remain at home. Confirm covered services, authorization, and current rates with SCDHHS.

Electronic Visit Verification (EVV)

South Carolina uses a state-mandated EVV model. For Medicaid in-home services, caregivers must check in and out using the state's designated system, AuthentiCare (Netsmart) — South Carolina does not offer an alternative-vendor pathway for these programs, so you use AuthentiCare's telephony or mobile app for every visit. SCDHHS has used EVV for its CLTC waivers for years. Confirm the current EVV system, service codes, and setup with SCDHHS before you configure anything, because EVV requirements change.

Wage and hour

South Carolina has no state minimum wage, so the federal Fair Labor Standards Act applies — at least the federal minimum wage and overtime at one and one-half times the regular rate over forty hours in a workweek, including compensable travel time between clients. Employ caregivers as W-2 employees.

Part 7 — Staying Compliant

- **Operate within scope** — non-medical in-home care only; refer clients needing skilled care.
- **Keep your records** — client service agreements, assessments, service plans, visit documentation, and personnel files — current, confidential, and available for DPH survey at your business address.
- **Respond to surveys** — cooperate, and submit a timely plan of correction for any deficiency; avoid repeat violations (a recurrence under the same section within 36 months).
- **Report abuse** — report suspected abuse, neglect, or exploitation of a vulnerable adult to South Carolina Adult Protective Services; call 911 if a client is in immediate danger.
- **Report disease** — report communicable and reportable conditions under DPH Regulation 61-20 to the county public health office.
- **Keep credentials current** — license, insurance, registration, and re-screening on schedule; notify DPH of changes in ownership, administrator, name, or location.

Part 8 — Your First 90 Days

1. Finalize your business, physical address, insurance, and bank account.
2. Adopt your Regulation 60-122 policy manual and forms.
3. Stand up your screening pipeline: background checks, abuse-registry coordination, drug testing, TB screening, and driver verification.
4. Submit your DPH application package and respond promptly to any questions.
5. Prepare for and pass any on-site survey.
6. Hire and train your first caregivers, documenting orientation and competency.
7. Enroll for payment (private pay first; Medicaid and AuthenticCare EVV if serving Healthy Connections).
8. Build two or three referral relationships and take your first clients — then deliver reliably.

Part 9 — Common Mistakes to Avoid

- Serving two or more hands-on clients before the license is issued.
- Confusing the In-Home Care Provider license (Reg 60-122) with skilled home health (Reg 61-77) — they are different licenses.
- Skipping or shortcutting the pre-employment and random five-panel drug testing South Carolina requires.
- Letting a caregiver start before background checks, abuse-registry coordination, TB screening, and training are complete and documented.
- Assigning transport duties without verifying the caregiver's license, insurance, and driving record.
- Misclassifying caregivers as 1099 contractors instead of W-2 employees.
- Using a third-party EVV tool for Medicaid visits — South Carolina mandates AuthentiCare with no alternative pathway.
- Paying for referrals of Medicaid clients, which violates the federal Anti-Kickback Statute.
- Using a PO box instead of a physical business address where records are kept.

Worked Example 1 — Columbia

Imagine you are opening in Columbia (the Midlands). Here is how the pieces connect in a real market. Verify every number before you rely on it.

- **Licensing:** You apply to the SC DPH Bureau of Health Facilities Licensing in Columbia and build your policies to Regulation 60-122.
- **Referral partners:** You introduce yourself to discharge planning at Prisma Health Richland Hospital (803-434-7000), Lexington Medical Center in West Columbia (803-791-2000), and Prisma Health Baptist Hospital (803-296-5010).
- **Aging network:** You connect with the SC Department on Aging (803-734-9900) and Senior Resources, Inc., the Midlands aging-services agency (803-252-7734), for the Community Choices Waiver and community referrals.
- **Getting paid:** You start with private-pay clients, then enroll with SC Healthy Connections and set up AuthentiCare EVV before taking Community Choices Waiver clients.

Worked Example 2 — Charleston

Now imagine opening in Charleston (the Lowcountry). Same license, different local network. Verify every number before you rely on it.

- **Licensing:** Same DPH In-Home Care Provider license under Regulation 60-122; your policies and screening pipeline are identical statewide.
- **Referral partners:** You build relationships with discharge planning at MUSC Health University Medical Center (843-792-2300), Roper Hospital (843-724-2000), and Bon Secours St. Francis Hospital (843-402-1000).
- **Aging network:** You connect with the Trident Area Agency on Aging in North Charleston (843-554-2275) and community hubs like the Waring Senior Center (843-402-1990) and Lowcountry Senior Center (843-990-5555).
- **Getting paid:** Private pay first, then SC Healthy Connections and the Community Choices Waiver with AuthentiCare EVV.

Who to Call — South Carolina Quick Directory

Verify each contact before you rely on it — numbers and offices change.

For...	Who to contact
License an in-home care provider	SC DPH, Bureau of Health Facilities Licensing (Healthcare Quality) · dph.sc.gov
The regulation and statute	S.C. Code Regs. 60-122 (formerly 61-122); Licensure of In-Home Care Providers Act, S.C. Code § 44-70-10 et seq.
Medicaid provider enrollment & waivers	SC Dept of Health & Human Services (SCDHHS) — Healthy Connections; Community Choices Waiver / CLTC · scdhhs.gov
Electronic Visit Verification	SCDHHS EVV — state-mandated AuthentiCare (Netsmart); verify current setup with SCDHHS
Criminal background checks	SC Law Enforcement Division (SLED) and FBI fingerprint-based checks (§ 44-7-2910)
Report abuse of a vulnerable adult	SC Adult Protective Services (SC Dept of Social Services); 911 if immediate danger
Aging network / Community Choices	SC Department on Aging (803-734-9900); your regional Area Agency on Aging
Business registration	SC Secretary of State (sos.sc.gov); IRS for an EIN
Communicable disease reporting	SC DPH, under Regulation 61-20, to your county public health office
Veteran in-home benefits	U.S. Dept of Veterans Affairs — 1-877-222-8387 · va.gov

Glossary

- **In-Home Care Provider (IHCP):** the South Carolina license for a non-medical home care agency, under Regulation 60-122.
- **Regulation 60-122:** the DPH “Standards for Licensing In-Home Care Providers” (formerly numbered 61-122).
- **DPH:** the SC Department of Public Health, the licensing authority (successor to DHEC for health-facility licensing since July 1, 2024).
- **Activities of daily living (ADLs):** walking, transferring, bathing, dressing, feeding, toileting, and preparing special diets.
- **Community Choices Waiver:** a South Carolina Medicaid Community Long Term Care waiver that can fund in-home personal care.
- **AuthentiCare:** the state-mandated electronic visit verification system for SC Medicaid in-home services.
- **Omnibus Adult Protection Act:** S.C. Code § 43-35-10 et seq., protecting vulnerable adults and governing abuse-registry coordination.
- **BAMT:** Blood Assay for Mycobacterium tuberculosis, a TB test used in screening.

Core Rules & References

- S.C. Code Regs. 60-122 (formerly 61-122) — Standards for Licensing In-Home Care Providers
- S.C. Code § 44-70-10 et seq. — Licensure of In-Home Care Providers Act (scope, licensure trigger)
- S.C. Code § 44-7-2910 et seq. — criminal record checks
- S.C. Code § 43-35-10 et seq. — Omnibus Adult Protection Act (abuse reporting and registries)
- S.C. DPH Regulation 61-20 — communicable and reportable disease reporting
- 21st Century Cures Act § 12006 — federal Electronic Visit Verification mandate
- Fair Labor Standards Act — federal minimum wage and overtime (South Carolina has no state minimum wage)

This guide is general educational information, not legal, tax, or accreditation advice. South Carolina rules, fees, programs, and phone numbers change — verify each with the SC DPH Bureau of Health Facilities Licensing and the relevant payer or authority before you rely on it.